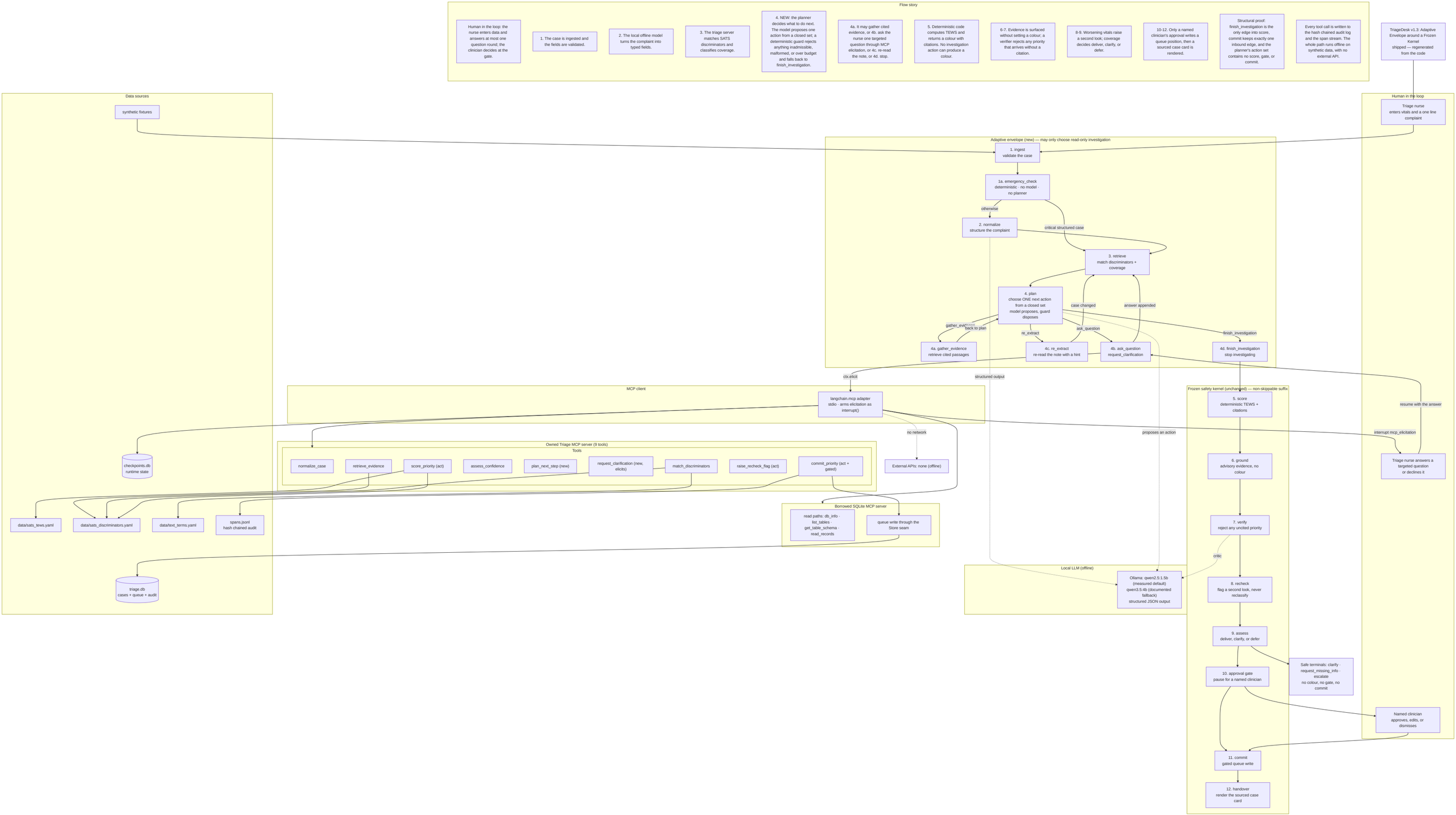

Flow story
Human in the loop: the nurse enters data and answers at most one question round; the clinician decides at the gate.
1. The case is ingested and the fields are validated.
2. The local offline model turns the complaint into typed fields.
3. The triage server matches SATS discriminators and classifies coverage.
4. NEW: the planner decides what to do next. The model proposes one action from a closed set; a deterministic guard rejects anything inadmissible, malformed, or over budget and falls back to finish_investigation.
4a. It may gather cited evidence, or 4b. ask the nurse one targeted question through MCP elicitation, or 4c. re-read the note, or 4d. stop.
5. Deterministic code computes TEWS and returns a colour with citations. No investigation action can produce a colour.
6–7. Evidence is surfaced without setting a colour; a verifier rejects any priority that arrives without a citation.
8-9. Worsening vitals raise a second look; coverage decides deliver, clarify, or defer.
10-12. Only a named clinician's approval writes a queue position, then a sourced case card is rendered.
Structural proof: finish_investigation is the only edge into score, commit keeps exactly one inbound edge, and the planner's action set contains no score, gate, or commit.
Every tool call is written to the hash chained audit log and the span stream. The whole path runs offline on synthetic data, with no external API.
TriageDesk v1.3: Adaptive Envelope around a Frozen Kernel
shipped — regenerated from the code
Data sources
synthetic fixtures
checkpoints.db
runtime state
data/sats_tews.yaml
data/sats_discriminators.yaml
data/text_terms.yaml
spans.jsonl
hash chained audit
triage.db
cases + queue + audit
Human in the loop
Triage nurse
enters vitals and a one line complaint
resume with the answer
interrupt mcp_elicitation
Triage nurse answers a targeted question or declines it
Adaptive envelope (new) — may only choose read-only investigation
1. ingest
validate the case
1a. emergency_check
deterministic · no model · no planner
otherwise
2. normalize
structure the complaint
critical structured case
3. retrieve
match discriminators + coverage
4. plan
choose ONE next action from a closed set
model proposes, guard disposes
case changed
answer appended
gather_evidence
back to plan
re_extract
ask_question
finish_investigation
4a. gather_evidence
retrieve cited passages
4c. re_extract
re-read the note with a hint
4b. ask_question
request_clarification
4d. finish_investigation
stop investigating
ctx.elicit
structured output
MCP client
langchain.mcp adapter
stdio · arms elicitation as interrupt()
no network
proposes an action
Owned Triage MCP server (9 tools)
Tools
normalize_case
retrieve_evidence
score_priority (act)
assess_confidence
plan_next_step (new)
request_clarification (new, elicits)
match_discriminators
raise_recheck_flag (act)
commit_priority (act + gated)
External APIs: none (offline)
Borrowed SQLite MCP server
read paths: db_info · list_tables · get_table_schema · read_records
queue write through the Store seam
Frozen safety kernel (unchanged) — non-skippable suffix
5. score
deterministic TEWS + citations
6. ground
advisory evidence, no colour
7. verify
reject any uncited priority
critic
8. recheck
flag a second look, never reclassify
9. assess
deliver, clarify, or defer
Safe terminals: clarify · request_missing_info · escalate
no colour, no gate, no commit
10. approval gate
pause for a named clinician
Named clinician approves, edits, or dismisses
11. commit
gated queue write
12. handover
render the sourced case card
Local LLM (offline)
Ollama: qwen2.5:1.5b (measured default)
qwen3.5:4b (documented fallback)
structured JSON output